

Back Pain - Female (Child)

Algorithm 1004 | Child (5-17 years) | Gender at birth: Female

Definition

- Back pain assessment decomposed from NHS.UK's published when-to-get-help guidance.
- Child patient.
- Female.

Initial assessment

1. [OPEN_TEXT] What is the person's exact age and sex, are they pregnant/recently postpartum, and where is the pain? Include trauma, cancer/immunocompromise, fever, anticoagulants, urinary symptoms, safeguarding concerns, and whether pain spreads to the chest, abdomen, or legs.
2. [DURATION] When did the pain start, and did it follow an injury or accident?
3. [YES_NO] Any numbness, tingling, or weakness in either leg?
4. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Disposition level 100

Is there numbness, tingling or weakness in BOTH legs, loss of feeling around the genitals or back passage, new difficulty controlling bladder or bowel, chest pain, or did the pain start after a serious accident such as a car crash or a fall from height?

NHS.UK back pain guidance lists bilateral leg symptoms, saddle numbness, new bladder/bowel changes, sexual dysfunction, concurrent chest pain, and a major-trauma trigger as its own Qatar 999/an emergency assessment service selected through the Qatar pathway approved by governance (GOVERNANCE_REQUIRED) criteria - possible cauda equina syndrome or major trauma.

1. Disposition level 70

Does the caller feel hot, cold, shivery, or generally unwell, OR is this severe pain that started suddenly and is quickly getting worse?

NHS.UK guidance flags fever/feeling unwell with back pain, and sudden severe or rapidly worsening pain, as needing a same-day clinical review through a Qatar pathway approved by governance (GOVERNANCE_REQUIRED) or the Qatar clinical-review pathway approved by governance (GOVERNANCE_REQUIRED) call.

1. Disposition level 50

Is this a patient aged 5 to 17 years with back pain and none of the emergency or urgent features above, including no fever or systemic illness, night or rest pain, unexplained weight loss, gait change, progressive weakness or numbness, bladder or bowel change, major trauma, or safeguarding concern?

NHS pediatric guidance identifies age, systemic illness, night/rest pain, weight loss, gait or neurological change, bladder/bowel symptoms, trauma, and safeguarding context as reasons for age-appropriate clinical assessment. The generated child pathway fails closed to in-person review rather than entering the adult self-care branch. Exact Qatar pediatric destination and timing remain GOVERNANCE_REQUIRED.

Advice

Qatar emergency back-pain response

- Call Qatar emergency services on 999 and do not drive for new bladder/bowel dysfunction, saddle numbness, progressive leg weakness, major trauma, collapse, or severe systemic illness. Keep the person still and do not make them stand or walk when spinal compromise is possible. If unresponsive and not breathing normally, follow 999 CPR instructions. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: worsening leg weakness; new saddle numbness or bladder/bowel change; reduced consciousness or abnormal breathing.

Urgent back pain review

- Arrange same-day or next-day medical review. Rest in a comfortable position and avoid heavy lifting until seen.
- Call Back If: numbness spreads; fever worsens.

Pediatric back-pain assessment required

- Arrange age-appropriate in-person pediatric assessment through the Qatar governance-approved pathway. Do not use the adult-derived self-care branch for a patient aged 5 to 17 years. Escalate immediately for major trauma, fever or severe illness, progressive weakness or numbness, saddle sensory loss, bladder/bowel change, chest pain, collapse, or worsening symptoms. Exact destination and timing remain GOVERNANCE_REQUIRED.
- Call Back If: fever or rapidly worsening pain; new weakness, numbness, gait change, or bladder/bowel change; major trauma, chest pain, collapse, or severe illness.

Routine back pain follow-up

- Book a routine primary-care appointment through the approved Qatar pathway. Continue gentle activity and use only governance-approved pain relief in the meantime.
- Call Back If: pain suddenly worsens; new leg numbness or weakness.

Governed care for uncomplicated adult back pain

- For an adult only, use gentle activity as tolerated and avoid prolonged bed rest or provoking strain. Wrapped cool or warm packs may be used without direct skin exposure. Medicine choice/dose requires age, weight, pregnancy, allergy, kidney/liver disease, ulcer/bleeding risk, anticoagulants, and current medicines under the approved Qatar pathway.
- Call Back If: pain does not improve; new numbness, weakness, fever, trauma, or bladder/bowel changes.

First aid

- Call Qatar emergency services on 999 and do not drive for new bladder/bowel dysfunction, saddle numbness, progressive leg weakness, major trauma, collapse, or severe systemic illness. Keep the person still and do not make them stand or walk when spinal compromise is possible. If unresponsive and not breathing normally, follow 999 CPR instructions. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Back pain", <https://www.nhs.uk/conditions/back-pain/> (page last reviewed 05 March 2026)
- Northern Care Alliance NHS Foundation Trust, "Paediatrics - Under 16 years Back Pain", <https://www.northern-care-alliance.nhs.uk/patient-information/patient-leaflets/paediatrics-under-16-years-back-pain>
- Sheffield Children's NHS Foundation Trust, Emergency Department Medical Guideline 4.24 "Back Pain", <https://www.sheffieldchildrens.nhs.uk/download/1721/acute-injury-and-orthopaedics/64216/4-24-back-pain.pdf>

Provenance

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